title: "Nutrition and Hydration Policy" version: "1.0" effectiveDate: "[Insert Date]" reviewDate: "[Insert Date – usually annually]" appliesTo: "All residents, staff, agency workers, and volunteers at [INSERT CARE HOME NAME]"

Nutrition and Hydration Policy

Policy Title: Nutrition and Hydration
Effective Date: [Insert Date]
Review Date: [Insert Date – usually annually]
Version: 1.0
Applies to: All residents, staff, agency workers, and volunteers at [INSERT CARE HOME NAME]

1. Policy Statement

At [INSERT CARE HOME NAME], we recognise the vital role of good nutrition and hydration in maintaining residents' health, independence, and quality of life. Malnutrition and dehydration can significantly impact wellbeing, increase the risk of illness, and reduce recovery from illness.

We aim to:

- Ensure all residents receive safe, nutritious, and enjoyable food and drink.
- Assess, monitor, and meet individual nutritional and hydration needs.
- Promote dignity, choice, and independence in eating and drinking.
- Comply with CQC Fundamental Standards, the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014, and NICE guidelines.

2. Purpose

This policy provides a framework for:

- Assessing residents' nutritional and hydration needs.
- Ensuring safe food and fluid provision.
- Monitoring intake and preventing malnutrition and dehydration.
- Training staff to support residents with eating and drinking.

3. Scope

This policy applies to:

- Residents – all individuals receiving care and support.
- Staff – all care, catering, and support staff.
- Environment – all dining areas, kitchens, and food preparation areas within the home.

4. Roles and Responsibilities

Registered Manager:

- Ensures policies and procedures are followed.
- Oversees training for staff in nutrition, hydration, and safe food handling.
- Ensures catering services meet residents' needs and preferences.

Nurses / Senior Care Staff:

- Carry out nutritional risk assessments (e.g., MUST – Malnutrition Universal Screening Tool).
- Develop and update individual care plans relating to nutrition and hydration.
- Monitor residents' weight, BMI, and fluid intake.
- Liaise with dietitians, GPs, and speech & language therapists (SLTs) as required.

All Care Staff:

- Support residents with eating and drinking according to their care plans.
- Record and monitor food and fluid intake.
- Report changes in appetite, swallowing, or weight promptly.

Catering Staff:

- Prepare and provide meals in line with residents' dietary requirements.
- Ensure safe food handling, storage, and preparation practices.
- Offer choice, variety, and culturally appropriate meals.

5. Nutritional Care

5.1 Assessment

- All residents will have a nutritional risk assessment on admission using the MUST tool.
- Assessments will be reviewed monthly or sooner if changes occur.

5.2 Care Planning

- Individualised care plans will detail dietary needs, preferences, allergies, and support required.
- Modified diets provided where necessary (e.g., soft, pureed, fortified).
- Plans reviewed regularly and updated after reassessments or incidents (e.g., weight loss).

5.3 Mealtime Support

- Residents offered support in a dignified and respectful manner.
- Adequate time allowed for eating and drinking.
- Independence promoted, with adaptive cutlery or aids provided as needed.
- Mealtime environment kept pleasant, calm, and sociable.

6. Hydration

- Residents will have access to fresh drinking water and fluids at all times.
- Staff will encourage regular fluid intake throughout the day.
- Fluid balance charts used for residents at risk of dehydration.
- Drinks tailored to individual preferences, including hot and cold options.

7. Specialist Support

- Residents with swallowing difficulties will be referred to a Speech & Language Therapist (SLT) for assessment and guidance.
- Dietitians consulted for complex nutritional needs or persistent weight issues.
- GP referrals made where medical conditions affect appetite, digestion, or hydration.

8. Monitoring and Review

- Resident weights monitored at least monthly (more frequently if risk identified).
- Food and fluid intake charts used for high-risk residents.
- Falls, infections, and hospital admissions reviewed for links to malnutrition or dehydration.
- Audit of menus and resident satisfaction conducted regularly.

9. Training

- All care staff trained in:
    - Nutrition and hydration awareness.
    - Supporting residents with eating and drinking.
    - Use of MUST and food/fluid charts.
- Catering staff trained in:
    - Special dietary requirements.
    - Safe food handling (Food Hygiene Level 2 minimum).

10. References

- Health and Social Care Act 2008 (Regulated Activities) Regulations 2014
- CQC Fundamental Standards – Regulation 14 (Meeting Nutritional and Hydration Needs)
- NICE Clinical Guideline CG32: Nutrition Support in Adults
- NICE Quality Standard QS24: Nutrition Support in Adults
- Food Standards Agency – Food Hygiene and Safety Guidance

11. Related Policies

- Infection Prevention & Control Policy
- Health & Safety Policy
- Risk Assessment Policy
- Mental Health & Wellbeing Policy (Staff & Residents)
- Equality, Diversity & Inclusion Policy
- Safeguarding Adults Policy

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